



Veterans Administration

VHA FAX TRANSMITTAL

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**Audie L. Murphy VA Hospital,
STVHCS**

7400 Merton Minter
San Antonio, TX 78229
210-617-5300

DATE:	Jan. 4, 2024
ATTN:	
FROM:	Gretchen Elizabeth Martinez
SUBJECT:	Referral

Note:

Good afternoon,
Please see referral for review and scheduling



U.S. Department
of Veterans Affairs

VA Form 10-7080 - Approved Referral For
Medical Care

Veteran Name: Fielder, Wells Thomas

Veteran ICN: 1013738462V232086

Veteran EDIPI: 1344342292

Veteran Date of Birth: 1955-02-24

Veteran Preferred Name:

Pronoun:

Pronoun Description:

Veteran Address: 3317 STATE HIGHWAY 111 N
EDNA, TX 77957

Veteran Phone Number: (361)782-2829

Veteran Mobile Phone Number (if Known): (361)782-1969

Veteran Business Phone Number (If Known): (999)999-9999

Veteran Email Address (If Known): billybobah1969@gmail.com

Referral Number: VA0033707427

Priority: Routine

Referral Issue Date: 2023-11-30

Expiration Date: PRELIMINARY 2024-05-28 (SEE BELOW)*

First Appointment Date: SUPPLY TO VA ASAP

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

VA Telephone Number: 210-949-3850

VA Fax Number:

Initial Community Care Provider/Facility: SOUTHEAST TEXAS DERMATOLOGY

Initial Provider Location: SOUTHEAST TEXAS DERMATOLOGY-115 Medical Dr ; Ste 202, Victoria, TX,
77904-207N00000X

Provider Name (if known): PATEL, AMIT

Community Provider NPI: 1346553120

Caregiver Type: Veteran does not rely on a caregiver

Caregiver Details:

***IN ORDER TO PROVIDE THE FULL DURATION OF THE STANDARDIZED EPISODE OF CARE (SEOC)
PLEASE CONTACT THE REFERRING VA FACILITY WITH A FIRST APPOINTMENT TO CALCULATE
A FINAL EXPIRATION DATE. CLAIM PAYMENTS ARE DEPENDENT UPON THESE DATES.**

Any claim related to this episode of care **MUST INCLUDE THE APPROVED REFERRAL NUMBER** as the
Referral Number or Prior Authorization number.

**Please see below for Additional VA Referring Facility Information, Billing Information
and Appointment/Provider Information.**

Pertinent Clinical Information

**Please view the Clinical Information in the VA Order section for more information related to the Original VA Order
Reason for Request.**

Chief Complaint: SKIN NEOPLASM UNCERTAIN BEHAVIOUR/SCALP

Patient History / Clinical Findings / Diagnosis (Co-Morbidities): 1X1CM, VARIED PIGMENT, BLEEDING, IRREG
BORDERS Is this referral for Service Connected condition? No Per VA PBM Formulary Process Management Policy
from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body
appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications

used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service-connected injury. Allergies and Medications (Optional): Allergies: Patient has answered NKA Medications: Active Outpatient Medications (including Supplies): Active Outpatient Medications Status

===== 1) ACETAMINOPHEN TAB 500MG TAKE TWO TABLETS BY MOUTH ACTIVE THREE TIMES A DAY AS NEEDED FOR PAIN OR FEVER**DO NOT EXCEED 4000MG ACETAMINOPHEN DAILY* 2) ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL INHALE 2 PUFFS ACTIVE BY INHALATION EVERY 4 HOURS AS NEEDED FOR BREATHING 3) ALBUTEROL SO4 0.083% INHL 3ML USE 3ML BY INHALATION ACTIVE (S) EVERY 6 HOURS AS NEEDED FOR BREATHING FOR RESCUE 4) ASPIRIN 81MG EC TAB TAKE ONE TABLET BY MOUTH EVERY ACTIVE DAY FOR HEART/TO THIN BLOOD 5) BusPIRone HCL TAB 10MG TAKE TWO TABLETS BY MOUTH ACTIVE TWICE A DAY AS NEEDED FOR ANXIETY 6) CHOLECALCIF 25MCG (D3-1,000UNIT) TAB TAKE TWO TABLETS ACTIVE BY MOUTH EVERY DAY FOR VITAMIN "D" DEFICIENCY 7) CYANOCOBALAMIN TAB 250 MCG TAKE TWO TABLETS BY MOUTH ACTIVE EVERY DAY FOR DIETARY SUPPLEMENT VITAMIN B12 8) DABIGATRAN ETEXILATE 150MG ORAL CAP UD TAKE ONE ACTIVE CAPSULE BY MOUTH TWICE A DAY FOR BLOOD CLOT 9) FOLIC ACID TAB 1MG TAKE ONE TABLET BY MOUTH EVERY DAY ACTIVE FOR NUTRITIONAL SUPPLEMENT 10) GABAPENTIN TAB 800MG TAKE ONE TABLET BY MOUTH EVERY ACTIVE MORNING AND TAKE ONE TABLET AT NOON AND TAKE ONE AND ONE-HALF TABLETS AT BEDTIME FOR PAIN * NOTE DOSE INCREASE* 11) LIDOCAINE 5% PATCH APPLY 1 PATCH TO SKIN EVERY DAY ACTIVE FOR PAIN**PATCH MAY BE WORN FOR UP TO 12 HOURS IN A 24 HOUR PERIOD** FOR RIB PAIN/ FRACTURES, ONLY LEAVE ON FOR 12 HOURS 12) LOSARTAN POTASSIUM 100MG TAB TAKE ONE-HALF TABLET BY ACTIVE MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE 13) METOPROLOL SUCC TAB 50MG SA TAKE ONE-HALF TABLET BY HOLD MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE AND HEART RATE CONTROL INCREASED DOSE 14) MOMETASONE 100MCG/ACTUAT 120D ORAL INHL INHALE 2 ACTIVE PUFFS BY MOUTH TWICE A DAY FOR CHRONIC OBSTRUCTED LUNG DISEASE 15) OLODATEROL/TIOTROP 2.5MCG/ACTUAT 60D INH INHALE 2 ACTIVE PUFFS BY MOUTH EVERY DAY FOR BREATHING. REPLACES SYMBICORT FOR MAINTENANCE INHALER/DAILY USE. NOT FOR RESCUE 16) TRAZODONE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH AT ACTIVE BEDTIME FOR SLEEP 17) VENLAFAXINE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH ACTIVE EVERY DAY FOR MOOD

Provisional Diagnosis: D485 Neoplasm of uncertain behavior of skin

Services Authorized

The VA Order Reason for Request is the official clinical order. This scope of services associated with the medical care for this authorization is found below. Necessary services that are not included must be requested using the Request for Services procedures. Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements.

Service Requested: Dermatology Comprehensive_PRCT SEOC 1.0.11

Category of Care: DERMATOLOGY

Procedural Overview - Standardized Episode of Care (SEOC)

Dermatology Comprehensive_PRCT SEOC 1.0.11 Duration: 180 Days

Description:

This authorization covers services associated with the specialty(s) identified for this episode of care, including all medical care listed below relevant to the referred care specified on the consult/referral order.

No.	Service/Procedure	Number Of Visits Authorized
1	Initial outpatient evaluation (including full body exam), treatment and follow-up visits for the referred condition on the consult/referral order	999
2	Diagnostic imaging relevant to the referred condition on the consult/referral order	999
3	Labs and pathology relevant to the referred condition on the consult/referral order	999
4	Procedures relevant to the referred condition on the consult/referral order including but not limited to: cryotherapy, biopsy (including excisional, shave, punch biopsies, etc.), excision, repair, excimer laser, photodynamic therapy, UVB therapy, debridement, medically necessary hair removal	999

Additional Information:

- * Please visit the VHA Storefront www.va.gov/COMMUNITYCARE/providers/index.asp for additional resources and requirements pertaining to the following:
- * Pharmacy prescribing requirements
- * Durable Medical Equipment (DME), Prosthetics, and Orthotics prescribing requirements
- * Precertification (PRCT) process requirements
- * Request for Services (RFS) requirements

REFER ALL QUESTIONS RELATED TO THIS APPROVAL TO THE ISSUING VA OFFICE

Referring VA Facility: Audie L. Murphy Memorial Veterans' Hospital

Station Number: 671

Telephone Number: 210-949-3850

Address: 7400 Merton Minter Boulevard SAN ANTONIO TX 78229

Referring Provider: LISA MORENO

Referring Provider NPI: 1275625295

Unique Consult No: 671_8304077

Program Authority: Authorized/Pre-authorized VA Referral (not otherwise specified) - 1703

Affiliation: Triwest

Network: CC Network 4

Appointments/Providers Assigned to the Referral

Provider/Facility Name	Provider/Facility Location	Appt Date	Appt Time	Telephone #
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Additional Service Information

Request for Services (RFS)

A Request for Services (RFS) is a provider-generated request for new or additional care outside the scope of the current approved referral/authorization. Provider should always submit the RFS directly to the authorizing VAMC, preferably via the HSRM portal. Providers should always submit an RFS on the same day it is determined it's needed and before delivering care, unless it is emergent care. In that case, the RFS can be submitted simultaneously.

How to Submit an RFS

VA prefers providers submit an RFS via the HSRM portal, available on the VA's website

- Go to the VA Storefront at https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp#RFS
 - Navigate to the link to the RFS form at the bottom of the section

Medical Records and Documents Requirements

Medical Records and documentation are required for all provider services. Providers are required to submit Medical documentation directly to the authorizing VAMC, preferably via upload to HSRM.

Billing and Other Referral Information

Submitting Claims

ANY CLAIMS RELATED TO THIS EPISODE OF CARE MUST BE SUBMITTED TO TRIWEST OR DELTA DENTAL AS APPROPRIATE AND INCLUDE THE APPROVED REFERRAL NUMBER.

Methods to submit claims:

Electronic Data Interchange (EDI):

Payer ID for Medical – TWVACCN

Payer ID for Dental – CDCA1

More information on how to submit claims can be found by visiting

https://www.va.gov/COMMUNITYCARE/revenue_ops/Veteran_Care_Claims.asp

Precertification

The Standardized Episode of Care (SEOC) referral you have accepted does not include services that require third-party payer (TPP) precertification.

Community Care Medical Policies

VA Community Care Medical Policies describe standard VA health care benefits for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies and supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>

Pharmacy

Drug Safety and Administration Requirements

• Must follow the VA National Protocol and clinical guidance for Esketamine or Ketamine administration. Ketamine treatments for mental health or for pain are not approved under a CCN referral. Prior to administration it is required the ordering and administering providers review the VA Protocol and Clinical Guidance found through the VA Formulary Search Tool available here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

Must follow the VA Opioid Safety Guidelines and complete the required Opioid Safety Training found here:

• <https://www.pbm.va.gov/apps/VANationalFormulary/>

• https://www.va.gov/COMMUNITYCARE/providers/EDU_Training.asp

CVS Caremark is the retail pharmacy network for Veterans' immediately needed or Urgent/Emergent prescriptions.

Immediate need prescriptions:

- Must follow the VA Urgent/Emergent Formulary which can be found at <http://www.pbm.va.gov/PBM/nationalformulary.asp>
- Prescription can only go up to a 14-day supply. No refills of the immediate need medication may be authorized.
- Only a seven-day supply for opioids, or up to the opioid prescribing limit allowed by State—whichever is less—may be authorized.

Immediate need prescription extending past 14 days:

The provider will need to send second prescription (beyond 14 days) to the referring VA medical facility's pharmacy for prescription fulfillment services.

Routine/maintenance prescriptions:

Must be sent to the referring VA medical facility's pharmacy

If you do not have the ability to electronically submit prescriptions to pharmacies, please contact the Community Care representative at the referring VA medical facility for their pharmacy fax number. Please refer to:

https://www.va.gov/COMMUNITYCARE/providers/Service_Requirements.asp for additional instructions related to prescriptions.

Clinical Information on the VA Order

Reason for Request:

Provider to receive results: L MORENO PA-C

Type of Service: Evaluation and Treatment

Chief Complaint: SKIN NEOPLASM UNCERTAIN BEHAVIOUR/SCALP

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

1X1CM, VARIED PIGMENT, BLEEDING, IRREG BORDERS

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the

use of drugs solely to improve normal physiologic function or to enhance body

appearance is generally not considered medically necessary and therefore are not

to be prescribed. Medications used for this purpose are considered cosmetic.

The use of medications for hair re-growth (e.g., minoxidil, finasteride, or

pimecrolimus) are considered cosmetic in nature and should not be prescribed

unless care is being provided in connection with the treatment of a service-

connected injury.

Allergies and Medications (Optional):

Allergies:

Patient has answered NKA

Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications Status

=====

1) ACETAMINOPHEN TAB 500MG TAKE TWO TABLETS BY MOUTH ACTIVE

THREE TIMES A DAY AS NEEDED FOR PAIN OR FEVER**DO

NOT EXCEED 4000MG ACETAMINOPHEN DAILY*

2) ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL INHALE 2 PUFFS ACTIVE

BY INHALATION EVERY 4 HOURS AS NEEDED FOR BREATHING

3) ALBUTEROL SO4 0.083% INHL 3ML USE 3ML BY INHALATION ACTIVE (S)

EVERY 6 HOURS AS NEEDED FOR BREATHING FOR RESCUE

4) ASPIRIN 81MG EC TAB TAKE ONE TABLET BY MOUTH EVERY ACTIVE

DAY FOR HEART/TO THIN BLOOD

- 5) BusPIRone HCL TAB 10MG TAKE TWO TABLETS BY MOUTH ACTIVE
TWICE A DAY AS NEEDED FOR ANXIETY
- 6) CHOLECALCIF 25MCG (D3-1,000UNIT) TAB TAKE TWO TABLETS ACTIVE
BY MOUTH EVERY DAY FOR VITAMIN "D" DEFICIENCY
- 7) CYANOCOBALAMIN TAB 250 MCG TAKE TWO TABLETS BY MOUTH ACTIVE
EVERY DAY FOR DIETARY SUPPLEMENT VITAMIN B12
- 8) DABIGATRAN ETEXILATE 150MG ORAL CAP UD TAKE ONE ACTIVE
CAPSULE BY MOUTH TWICE A DAY FOR BLOOD CLOT
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FOR NUTRITIONAL SUPPLEMENT
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MORNING AND TAKE ONE TABLET AT NOON AND TAKE ONE
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DOSE INCREASE*
- 11) LIDOCAINE 5% PATCH APPLY 1 PATCH TO SKIN EVERY DAY ACTIVE
FOR PAIN**PATCH MAY BE WORN FOR UP TO 12 HOURS IN A
24 HOUR PERIOD** FOR RIB PAIN/FRACTURES, ONLY LEAVE
ON FOR 12 HOURS
- 12) LOSARTAN POTASSIUM 100MG TAB TAKE ONE-HALF TABLET BY ACTIVE
MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE
- 13) METOPROLOL SUCC TAB 50MG SA TAKE ONE-HALF TABLET BY HOLD
MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE AND HEART
RATE CONTROL INCREASED DOSE
- 14) MOMETASONE 100MCG/ACTUAT 120D ORAL INHL INHALE 2 ACTIVE
PUFFS BY MOUTH TWICE A DAY FOR CHRONIC OBSTRUCTED
LUNG DISEASE
- 15) OLODATEROL/TIOTROP 2.5MCG/ACTUAT 60D INH INHALE 2 ACTIVE
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SYMBICORT FOR MAINTENANCE INHALER/DAILY USE: NOT
FOR RESCUE
- 16) TRAZODONE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH AT ACTIVE
BEDTIME FOR SLEEP
- 17) VENLAFAXINE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH ACTIVE
EVERY DAY FOR MOOD

**This 10-7080 - Approved Referral For Medical Care was generated on 01/04/2024 changes made to the referral after
this date are not reflected on the form.



U.S. Department of Veterans Affairs

Veterans Health Administration
Office of Community Care

Health Share Referral Manager (HSRM): VA's Secure Online Portal for Managing Referrals and Authorizations

Why are community providers across the country excited to use HSRM?

- ◆ **Facilitates Health Information Exchange (HIE)** between community providers and VA via a unified platform
- ◆ Conveniently **organizes all active referrals** in one centralized location
- ◆ **Reduces time spent** waiting for fax, phone or email contact
- ◆ Allows community providers to **request authorization** for additional services or additional time to provide services
- ◆ Promotes **reduced turnaround time** for authorizations and reimbursement

HSRM Account Creation For Community Providers

- ◆ **STEP 1: Training:** Each staff member attends virtual training, completes eLearning lessons, or reviews one of the training guides
- ◆ **STEP 2: ID.me:** Each staff member creates an ID.me account and verifies their identity at <https://www.id.me/>
- ◆ **STEP 3: Submit EUT:** One facility point of contact (POC) fills out the End User Tracker (EUT), then sends it to HSRMSupport@va.gov
- ◆ **STEP 4: Receive Accounts:** Help Desk creates accounts in HSRM, then provides confirmation of account creation to the facility POC
- ◆ **STEP 5: Log Into HSRM:** Each staff member logs into HSRM at <https://ccracommunity.va.gov>
- ◆ Once these steps are complete, contact the VA Medical Center(s) you work with to let them know you have access to HSRM and to discuss your transition to HSRM

HSRM Registration Resources

- ◆ HSRM Support Points of Contact: <https://www.va.gov/COMMUNITYCARE/providers/HSRM-POC-List.asp>
- ◆ ID.me Registration Instructions: https://www.va.gov/COMMUNITYCARE/docs/pubfiles/factsheets/FactSheet_26-06.pdf#
- ◆ End User Tracker
 - End User Tracker Template: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_End_User_Tracker.xlsx#
 - Send completed End User Tracker to HSRMSupport@va.gov
- ◆ HSRM Login: <https://ccracommunity.va.gov>

HSRM Educational Resources

- ◆ Live Virtual Training: https://www.train.org/virginia/course/1082953/live_event; please visit link to find available dates and to register in advance
- ◆ HSRM eLearning lessons: Log into <https://www.train.org/vha/welcome>, then search the course catalogue for "HealthShare Referral Manager"
- ◆ HSRM User Guide: https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_User_Guide.pdf
- ◆ HSRM Quick Reference Guide (QRG): https://www.va.gov/COMMUNITYCARE/docs/providers/HSRM_Quick_Reference_Guide.pdf#
- ◆ Additional information about HSRM: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp
- ◆ HSRM Help Desk
Phone: 1-844-293-2272, Email: HSRMSupport@va.gov, Hours: Monday - Friday 7:30 a.m. - 5 p.m. Eastern Time



Veterans Health Administration

VHA Medical Documentation

Document Created: 01/04/2024 20:15

The documents accompanying this transmission contain confidential health information that is legally privileged. This information is intended only for the use of the individual or entity providing care to this Veteran. The authorized recipient of this information is prohibited from disclosing this information to any other party unless required to do so by law or regulation and is required to destroy the information after its stated need has been fulfilled and in accordance with agency destruction and retention requirements. If you are not the intended recipient, you are hereby notified that any disclosure, copying, or distribution is strictly prohibited as this information is protected by Federal Privacy law (e.g., HIPAA Privacy Rule). If you have received this information in error, please notify the sender immediately and arrange for the return or destruction of these documents.

AUDIE L MURPHY MEMORIAL VAMC 7400 Merton Minter Blvd, San Antonio, Texas 78229-4404

VA Referrals phone: 210-949-3850 Fax:

Point of Contact (POC): MARTINEZ, GRETCHEN

POC Phone: 2109493850 Fax:

Patient Name: FIELDER,WELLS THOMAS DOB: 02/24/1955

3317 STATE HIGHWAY 111 N
EDNA, TEXAS 77957

Phone (residential): (361)782-2829
Phone (mobile): (361)782-1969

Referral Type: Community Care
Network

Next of Kin contact information

FIELDER,NORMA

Address (Next Of Kin)
3317 ST HWY 111 N
EDNA, TEXAS 77957

Phone: (361)782-2829
Phone (work): NONE

CONSULTS

Current PC Provider: MORENO,LISA ANN

Current PC Team: VICTORIA GREEN TEAM *WH* *CRH*

Current Pat. Status: Outpatient

Primary Eligibility: SERVICE CONNECTED 50% to 100%

UCID: 671_8304077

Patient Type:

OEF/OIF: NO

Order Information

To Service: COMMUNITY CARE-DERMATOLOGY

Attention:

From Service: VOP-V17 PACT PA PT CVT

Requesting Provider: MORENO,LISA ANN

Service is to be rendered on an OUTPATIENT basis

Place: Consultant's choice

Urgency: Routine

Clinically Ind. Date: 2023-11-29 00:00:00

DST ID:

Orderable Item:

Consult: Consult

Provisional Diagnosis: Neoplasm Of Uncertain Behavior Of Skin (ICD-10-CM D48.5)

Reason For Request:

Provider to receive results: L MORENO PA-C

Type of Service: Evaluation and Treatment

Chief Complaint: SKIN NEOPLASM UNCERTAIN BEHAVIOUR/SCALP

Patient History / Clinical Findings / Diagnosis (Co-Morbidities):

1X1CM, VARIED PIGMENT, BLEEDING, IRREG BORDERS

Is this referral for Service Connected condition? No

Per VA PBM Formulary Process Management Policy from VHA Handbook 1108.08, the use of drugs solely to improve normal physiologic function or to enhance body appearance is generally not considered medically necessary and therefore are not to be prescribed. Medications used for this purpose are considered cosmetic. The use of medications for hair re-growth (e.g., minoxidil, finasteride, or pimecrolimus) are considered cosmetic in nature and should not be prescribed unless care is being provided in connection with the treatment of a service- connected injury. Allergies and Medications (Optional): Allergies: Patient has answered NKA Medications:

Active Outpatient Medications (including Supplies):

Active Outpatient Medications	Status
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THREE TIMES A DAY AS NEEDED FOR PAIN OR FEVER**DO	
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 BEDTIME FOR SLEEP 17) VENLAFAXINE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH ACTIVE
 EVERY DAY FOR MOOD

Inter-facility Information

AUDIE L. MURPHY MEMORIAL HOSP

Status: ACTIVE

Last Action: RECEIVED

Facility

Activity Date/Time/Zone Responsible Person Entered By

CPRS RELEASED ORDER 11/29/23 14:51 MORENO,LISA ANN MORENO,LISA ANN
 ADDED COMMENT 11/29/23 00:00 MORENO,LISA ANN MORENO,LISA ANN
 RECEIVED 11/30/23 08:04 LARS,SUSAN M LARS,SUSAN M

Note: TIME ZONE is local if not indicated

No local TIU results or Medicine results available for this consult

=====

===== END =====

PROBLEM LIST

Sensitive Diagnoses

Problem	Code
Alcohol dependence with other alcohol-induced diso	F10.288
Cannabis abuse, uncomplicated	F12.10

Other Diagnoses

Problem	Code
Abnormal results of liver function studies	R94.5
Abnormal results of liver function studies	R94.5
Alcoholic fatty liver	K70.0
Alcoholic gastritis without bleeding	K29.20
Anxiety disorder, unspecified	F41.9
Cerebral infarction, unspecified	I63.9
Cervicalgia	M54.2
Chronic embolism and thombos unsp deep vn unsp low	I82.509
Chronic embolism and thrombosis of deep veins of r	I82.721
Chronic obstructive pulmonary disease, unspecified	J44.9
Diarrhea, unspecified	R19.7
Disorders of esophagus in diseases classified else	K23.
Duodenitis without bleeding	K29.80
Dysphagia, unspecified	R13.10
Essential (primary) hypertension	I10.
Gastro-esophageal reflux dis with esophagitis, wit	K21.00
GOUT NOS	274.9

Gross hematuria	R31.0
HYPERLIPIDEMIA NEC/NOS	272.4
Long term (current) use of anticoagulants	Z79.01
Low back pain	M54.5
Malignant tumor of renal pelvis	R69.
Narcotic drug user	R69.
Nicotine dependence, cigarettes, uncomplicated	F17.210
Nontoxic single thyroid nodule	E04.1
Other abnormality of red blood cells	R71.8
Other intervertebral disc degeneration, lumbar reg	M51.36
Other recurrent depressive disorders	F33.8
Pain in left hip	M25.552
Polyp of colon	K63.5
Radiculopathy, lumbosacral region	M54.17
TENOSYNOV HAND/WRIST NEC	727.05
Ventral hernia without obstruction or gangrene	K43.9

MEDICATIONS

100 most recent outpatient medications released by VA to Veteran in the last 6 months

Medication Name and Dose	Quantity	Refill Number	Issue and Fill Date	Status
ACETAMINOPHEN TAB 500MG TAKE TWO TABLETS BY MOUTH THREE TIMES A DAY AS NEEDED FOR PAIN OR FEVER**DO NOT EXCEED 4000MG ACETAMINOPHEN DAILY*	Qty:200	Fill: 1 of 5	Orig: 2023-11-29 Last: 2023-11-29	ACTIVE
ALBUTEROL SO4 0.083% INHL 3ML USE 3ML BY INHALATION EVERY 6 HOURS AS NEEDED FOR BREATHING FOR RESCUE	Qty:30	Fill: 3 of 6	Orig: 2023-07-25 Last: 2023-12-12	ACTIVE
ASPIRIN 81MG EC TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR HEART/TO THIN BLOOD	Qty:120	Fill: 1 of 3	Orig: 2023-11-29 Last: 2023-11-29	ACTIVE
BusPIRone HCL TAB 10MG TAKE TWO TABLETS BY MOUTH TWICE A DAY AS NEEDED FOR ANXIETY	Qty:120	Fill: 1 of 5	Orig: 2023-11-29 Last: 2023-12-19	ACTIVE
CYANOCOBALAMIN TAB 250 MCG TAKE TWO TABLETS BY MOUTH EVERY DAY FOR DIETARY SUPPLEMENT VITAMIN B12	Qty:130	Fill: 1 of 1	Orig: 2023-07-25 Last: 2023-08-27	ACTIVE
DABIGATRAN ETEXILATE 150MG ORAL CAP UD TAKE ONE CAPSULE BY MOUTH TWICE A DAY FOR BLOOD CLOT	Qty:180	Fill: 1 of 3	Orig: 2023-07-20 Last: 2023-12-19	ACTIVE
FOLIC ACID TAB 1MG TAKE ONE TABLET BY MOUTH EVERY DAY FOR NUTRITIONAL SUPPLEMENT	Qty:90	Fill: 1 of 3	Orig: 2023-07-25 Last: 2023-10-13	ACTIVE
GABAPENTIN TAB 800MG TAKE ONE TABLET BY MOUTH THREE TIMES A DAY FOR NERVE PAIN FOR PAIN * NOTE DOSE INCREASE* REDUCED DOSE	Qty:270	Fill: 1 of 3	Orig: 2023-11-29 Last: 2023-11-29	ACTIVE
LIDOCAINE 5% PATCH APPLY 1 PATCH TO SKIN EVERY DAY FOR PAIN**PATCH MAY BE WORN FOR UP TO 12 HOURS IN A 24 HOUR PERIOD** FOR RIB PAIN/FRACTURES, ONLY LEAVE ON FOR 12 HOURS	Qty:30	Fill: 2 of 11	Orig: 2023-07-25 Last: 2023-09-13	ACTIVE

LOSARTAN POTASSIUM 50MG TAB TAKE ONE-HALF TABLET BY MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE NOTE CHANGE IN DOSAGE STRENGTH (REDUCED DOSE).	Qty:45	Fill: 1 of 1	Orig: 2023-11-30 Last: 2023-11-30	ACTIVE
MOMETASONE 100MCG/ACTUAT 120D ORAL INHL INHALE 2 PUFFS BY MOUTH TWICE A DAY FOR CHRONIC OBSTRUCTED LUNG DISEASE	Qty:2	Fill: 1 of 3	Orig: 2023-11-29 Last: 2023-11-29	ACTIVE
OMEPRAZOLE 20MG EC CAP TAKE ONE CAPSULE BY MOUTH EVERY DAY FOR REFLUX. TAKE 30 TO 60 MINUTES BEFORE MEAL.	Qty:90	Fill: 1 of 3	Orig: 2023-11-29 Last: 2023-11-29	ACTIVE
LOSARTAN POTASSIUM 100MG TAB TAKE ONE-HALF TABLET BY MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE	Qty:45	Fill: 1 of 3	Orig: 2023-08-04 Last: 2023-08-07	DISCONTINUED (EDIT)
ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL INHALE 2 PUFFS BY INHALATION EVERY 4 HOURS AS NEEDED FOR BREATHING	Qty:2	Fill: 3 of 5	Orig: 2023-07-25 Last: 2024-01-11	SUSPENDED
METOPROLOL SUCCINATE 100MG SA TAB TAKE ONE-HALF TABLET BY MOUTH EVERY DAY FOR VENTRICULAR RATE CONTROL INCREASED DOSE, ATTENTION TO DIRECTIONS	Qty:45	Fill: 1 of 3	Orig: 2023-11-30 Last: 2024-02-18	SUSPENDED
OLODATEROL/TIOTROP 2.5MCG/ACTUAT 60D INH INHALE 2 PUFFS BY MOUTH EVERY DAY FOR BREATHING. REPLACES SYMBICORT FOR MAINTENANCE INHALER/DAILY USE. NOT FOR RESCUE	Qty:2	Fill: 1 of 5	Orig: 2023-11-29 Last: 2024-01-11	SUSPENDED
TRAZODONE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH AT BEDTIME FOR SLEEP	Qty:180	Fill: 2 of 3	Orig: 2023-07-25 Last: 2024-01-11	SUSPENDED
VENLAFAXINE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH EVERY DAY FOR MOOD	Qty:180	Fill: 2 of 3	Orig: 2023-07-25 Last: 2024-01-11	SUSPENDED

ALLERGIES

No documented allergies

PROGRESS NOTES

LOCAL TITLE: VOPC TELEHEALTH ESTABLISHED

STANDARD TITLE: TELEHEALTH NOTE

DATE OF NOTE: NOV 29, 2023@14:22 ENTRY DATE: NOV 29, 2023@07:11:49

AUTHOR: MORENO, LISA ANN EXP COSIGNER:

URGENCY: STATUS: COMPLETED

*** VOPC TELEHEALTH ESTABLISHED Has ADDENDA ***

*** PATIENT CONSENTED TO CLINIC VISIT VIA TELEHEALTH ***

Telehealth Disclosure: Visit conducted by clinical video telehealth.
Patient verbal consent obtained.

Veteran was identified using full name, SSN, and date of birth.

CHIEF COMPLAINT: Evaluation of chronic medical problems.

HISTORY OF PRESENT ILLNESS:

*** no lab completed prior, lab drawn today

- Vet states he has a skin rash around left eye that has been there several months and would like evaluated.

- hosp: Jul 26, 2023 Hospital: CMC Victoria TX, Chief complaint: Hyponatremia, hypoglycemia Primary Diagnosis: UTI

- discharged on O2, h/o COPD/smoker

Summary: Treated for hypotension, hyponatremia. BP 75/56, HR 104, UTI, resp failure with hypoxia related to bronchitis/emphysema

1. Oxygen saturation while at rest on room air: SpO2 88%

- Aug 11, 2023 Hospital: CITIZENS MEDICAL CENTER

Chief complaint: FALL, WEAK, BLOOD IN URINE, HIT HEAD

- CC GI consult cancelled due to failed scheduling efforts/was hospitalized. h/o GERD/gastritis/duodenitis/DYSPLASIA/h/o colon polyps 2017, elevated LFTs/fatty infiltration/fibrosis per FIB 4/ETOH abuse

- urothelial carcinoma/rt renal pelvis (new dx), s/p rt nephrectomy (Dr. New)

- reports metoprolol dose reduce to 25mg in hosp, not checking home BPs. Gabapentin dose reduced to 800mg tid per vet s/p nephrectomy.

ALLERGIES:

Patient has answered NKA

MEDICATIONS:

Active Outpatient Medications (including Supplies):

ACETAMINOPHEN TAB 500MG TAKE TWO TABLETS BY MOUTH THREE TIMES A DAY AS NEEDED FOR PAIN OR FEVER**DO NOT EXCEED 4000MG ACETAMINOPHEN DAILY* ACTIVE

ALBUTEROL 90MCG (CFC-F) 200D ORAL INHL INHALE 2 PUFFS BY INHALATION EVERY 4 HOURS AS NEEDED FOR BREATHING ACTIVE

ALBUTEROL SO4 0.083% INHL 3ML USE 3ML BY INHALATION EVERY 6 HOURS AS NEEDED FOR BREATHING FOR RESCUE ACTIVE (S)

ASPIRIN 81MG EC TAB TAKE ONE TABLET BY MOUTH EVERY DAY FOR HEART/TO THIN BLOOD ACTIVE

BusPIRone HCL TAB 10MG TAKE TWO TABLETS BY MOUTH TWICE A DAY AS NEEDED FOR ANXIETY ACTIVE

CHOLECALCIF 25MCG (D3-1,000UNIT) TAB TAKE TWO TABLETS BY MOUTH EVERY DAY FOR VITAMIN "D" DEFICIENCY ACTIVE

CYANOCOBALAMIN TAB 250 MCG TAKE TWO TABLETS BY MOUTH EVERY DAY FOR DIETARY SUPPLEMENT VITAMIN B12 ACTIVE

DABIGATRAN ETEXILATE 150MG ORAL CAP UD TAKE ONE CAPSULE BY MOUTH TWICE A DAY FOR BLOOD CLOT ACTIVE

FOLIC ACID TAB 1MG TAKE ONE TABLET BY MOUTH EVERY DAY FOR NUTRITIONAL SUPPLEMENT ACTIVE

GABAPENTIN TAB 800MG TAKE ONE TABLET BY MOUTH EVERY MORNING AND TAKE ONE TABLET AT NOON AND TAKE ONE AND ONE-HALF TABLETS AT BEDTIME FOR PAIN * NOTE DOSE INCREASE* ACTIVE

*** reduced to 800mg tid per vet, in hosp

LIDOCAINE 5% PATCH APPLY 1 PATCH TO SKIN EVERY DAY FOR PAIN**PATCH MAY BE WORN FOR UP TO 12 HOURS IN A 24 HOUR PERIOD** FOR RIB PAIN/FRACTURES, ONLY LEAVE ON FOR 12 HOURS ACTIVE

LOSARTAN POTASSIUM 100MG TAB TAKE ONE-HALF TABLET BY MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE ACTIVE

METOPROLOL SUCC TAB 50MG SA TAKE ONE-HALF TABLET BY MOUTH EVERY DAY FOR HIGH BLOOD PRESSURE AND HEART RATE CONTROL INCREASED DOSE HOLD

MOMETASONE 100MCG/ACTUAT 120D ORAL INHL INHALE 2 PUFFS BY MOUTH TWICE A DAY FOR CHRONIC OBSTRUCTED LUNG DISEASE
OLODATEROL/TIOTROP 2.5MCG/ACTUAT 60D INH INHALE 2 PUFFS BY MOUTH EVERY DAY FOR BREATHING. REPLACES SYMBICORT FOR MAINTENANCE INHALER/DAILY USE. NOT FOR RESCUE
TRAZODONE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH AT BEDTIME FOR SLEEP
VENLAFAXINE HCL TAB 100 MG TAKE TWO TABLETS BY MOUTH EVERY DAY FOR MOOD

OTC/HERBAL MEDICATIONS:

NON-VA PRESCRIPTION MEDICATIONS:

Medication Reconciliation:

Medication Reconciliation includes comparing the VA list of all medications to the patient's/caregiver's medication information:

I have reviewed this list of medications with the Patient/caregiver and medication reconciliation was performed. Discrepancies were noted with ***** above.

PHYSICAL EXAM: (portions of exam completed by trained CVT Telepresenter under provider guidance/direction)

Vitals: SEE COVER SHEET

GENERAL: No acute distress, well developed, well nourished, ambulatory

HEENT: Normocephalic, atraumatic, extraocular movement intact bilaterally.

CHEST/LUNGS: Rhonci throughout. Occ. exp wheeze.

HEART/VASCULAR: Regular rate, irregular rhythm, no rub, no gallop, no murmur
skin: posterior scalp- 1x1cm scaly lesion with irregular borders and scabbed appearance. left eye, lateral aspect- redness without pigmentation

NEUROLOGIC: Mental status: alert and oriented. No significant abnormality of gait appreciated.

LABS REVIEWED: none

ASSESSMENT/PLAN:

1. severe chronic COPD/Tobacco dependency, chronic resp. failure requiring O2 - CC pulmonary consult pending. Cont. mometasone (ICS) + olodaterol/tiotropium (LAMA/LABA) for maintenance. Continue albuterol inhaler/nebs for rescue. Smoking cessation recommended, down to 4cig/day from 2ppd, declines cessation aids.

- CHEST CT (Citizen's) 04/12/22: Mild emphysematous changes. No focal consolidation. Mild diffuse bronchial wall thickening which may be seen in bronchitis, asthma, or smoking related change.

- PFTs (Citizen's Med Center) 06/02/22: Moderately severe partially reversible airflow obstruction. Mild air trapping present. The DLCO is moderately decreased. Saw toothing of the FVC tracing can be associated with OSA, needs clinical correlation. (Dr. Lompart)

2. urothelial carcinoma/rt renal pelvis s/p recent rt. nephrectomy- f/u CC urology/Dr. New.

- smoker

3. a. fib/chronic dyspnea, anti-coagulated, rate elevated, asymptomatic- no change. f/u Dr. Tillman/Cardiology, cont. metoprolol SA (dose reduced during hosp) + apixaban.
- chronic anti-coag due to UE DVT
4. DDD l spine/low back pain/bilat LE radiculopathy s/p L2-S1 fusion
08/26/2021- Cont. acetaminophen 1000mg tid, cont. gabapentin (GFR pending, s/p nephrectomy, dose reduction may be required). narcotics not a good option with h/o falls/ETOH abuse. Veteran
- declines to f/u with neurosurgery/Dr. Macias VA comm care.
5. essential hypertension/h/o symptomatic hypotension, BP/HR elevated in clinic
- re-initiate HT and f/u pharm med mang. f/u PaCT RN call to assess home BP/HR, veteran struggled with walking today due to pain which may cause elevation.
6. GERD/gastritis/duodenitis/dysphagia (resolved)/h/o colon polyps 2017 - CC GI for EGD/cscope and colonoscopy. Cont omeprazole 20mg daily. Avoid NSAIDS on anti-coag. At risk for esoph. varices with liver fibrosis.
- No prior EGD.
- tobacco use
7. elevated LFTs/fatty infiltration/fibrosis per FIB 4/ETOH abuse - cc GI CONSULT
- no h/o cholecystectomy
- US HEPATIC 03/10/22 (CITIZEN'S): 1. no definite evidence of acute or chronic gallbladder disease. 2. fatty infiltration of liver. 3. simple-appearing cyst rt. kidney.
- FIB 4 SCORE: 3.34 pts METAVIR stage F3-F4 likely. Advanced fibrosis. Approx fibrosis stage: Ishak 4-6.
8. Squamous cell carcinoma in situ of left vocal cord s/p surgery 8/14/2020 - NO f/u PLANNED WITH CC ENT/Dr. Bickford
9. Recurrent DVT bilat/UE, h/o CVA- patient will remain on anticoagulation indefinitely, currently is on Pradaxa + ASA. He remains enrolled in the anticoagulation clinic. Veteran is aware to seek emergent care with signs of DVT.
- Anticoagulant Hx
6/2014 - 1st provoked DVT - anticoagulated w/Rivaroxaban x 9 months
4/215 - dx with left MCA stroke - Plavix initiated
3/2017 - dx with 2nd DVT Right-- subclavian artery occlusion-- initiated on non VA apix, then subsequently changed to VA preferred dabigatran
10. vitamin D deficiency- replete, cont vit D
11. PTSD - declines f/u with VA MH Dr John. States sx well-controlled on venlafaxine, trazodone, and BuSpar.
12. skin neoplasm of uncertain behav, scalp - CC derm consult
- HCM: colonoscopy (non-VA) 2017

IMAGING:

CONSULTS: CC GI, CC DERM

Return to Clinic: F2F w lab, 6mo

Time spent on this face to face encounter: 30 minutes with at least 50% of

that time spent on counseling, teaching, and collaboration of care.

/es/ LISA ANN MORENO
PHYSICIAN ASSISTANT
Signed: 11/29/2023 16:33

11/30/2023 ADDENDUM STATUS: COMPLETED
Collection time: Nov 29, 2023@14:11

Test Name	Result	Units	Range
GLUCOSE	91	mg/dL	70 - 105
UREA NITROGEN	12	mg/dL	7 - 25
CREATININE	1.5 H	mg/dL	.7 - 1.3
SODIUM	137	mmol/L	136 - 145
POTASSIUM	5.6 H	mmol/L	3.5 - 5.1
CHLORIDE	98	mmol/L	98 - 107
CO2	30.0	mmol/L	21 - 31
ANION GAP	9.0		6 - 15
CALCIUM	9.8	mg/dl	8.6 - 10.3
PROTEIN,TOTAL	7.4	g/dL	6.4 - 8.9
ALBUMIN	3.5	g/dl	3.5 - 5.7
ALKALINE PHOSPHATASE	174 H	U/L	34 - 104
TOT. BILIRUBIN	0.5		0.3 - 1.0
AST	29	IU/L	13 - 39
ALT	13	IU/L	7 - 52
TIBC	343.00	ug/dl	225 - 400
IRON	123	ug/dL	50 - 212
IRON SATURATION	36.0	%	20 - 55
EGFR CKD EPI	50		

Kidney function is reduced as expected s/p rt nephrectomy.
Hyperkalemia/BP and HR elevated in clinic- PACT RN please have veteran HOLD
losartan for one day and then REDUCE from 50mg to 25mg. INCREASE metoprolol from
25mg to 50mg (take full tab). Have vet check BP/HR at home at rest and alert
team if BP > 160/90 or < 100/60 and if HR at rest < 50. Schedule f/u RN phone
appt with f/u lab/K within 3 wks.

/es/ LISA ANN MORENO
PHYSICIAN ASSISTANT
Signed: 11/30/2023 06:04

Receipt Acknowledged By:
11/30/2023 09:08 /es/ Karen Jennings, RN, BSN
Staff Nurse

11/30/2023 ADDENDUM STATUS: COMPLETED
Call to patient per PCP and relayed what she desire he know. RN did ask him to
write down the treatment plan which he shares he did. He agrees with nursing
telephone follow up in 3 weeks. RTC already placed.

/es/ Karen Jennings, RN, BSN
Staff Nurse
Signed: 11/30/2023 09:11

12/12/2023 ADDENDUM STATUS: COMPLETED
- US HEPATIC (CITIZENS): 12/11/23- Mild hepatic steatosis. Nonvisualization of
pancreas. No gallstones. No other significant finding.

To:

From: Gretchen Elizabeth Martinez

01/04/24 02:26 PM

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/es/ LISA ANN MORENO
PHYSICIAN ASSISTANT
Signed: 12/12/2023 13:23

 Department of Veterans Affairs		REQUEST FOR SERVICES (RFS) FORM	
PREVIOUS AUTHORIZATION NUMBER:		NOTE: The Request for Services (RFS) Form 10-10172 must be submitted via an approved method (HSRM, Electronic fax, Direct Messaging, traditional fax, or mail). Completion of this form is REQUIRED and MUST BE SIGNED by the requesting provider for further care to be rendered to a Veteran patient.	
TODAY'S DATE (MM/DD/YYYY):			
SECTION I: VETERAN INFORMATION			
1. VETERAN'S LEGAL FULL NAME (First, MI, Last):		2. DOB (MM/DD/YYYY):	
3. VA FACILITY: STVHCS/Pink Team - Team Fax: (210) 443-0301		4. VA LOCATION: San Antonio, TX Ph: (210) 949-3850	
SECTION II: ORDERING PROVIDER INFORMATION			
5. REQUESTING PROVIDER'S NAME:		6. NPI #:	
7. SPECIALTY:			
8. OFFICE NAME & ADDRESS:			
9. SECURE EMAIL ADDRESS:			
10. PHONE NUMBER:		11. FAX NUMBER:	
☐ 12. INDIAN HEALTH SERVICES (IHS) PROVIDER?			
SECTION III: TYPE OF CARE REQUEST			
13. PLEASE INDICATE CLINICAL URGENCY (Urgent care is only applicable for requests that require less than 3 days to process. If care is needed within 48 hours or if Veteran is at risk for Suicide/Homicide, please call the VA directly on the same day as completed RFS form submission. Do NOT mark urgent for administrative urgency): ☐ ROUTINE ☐ URGENT			
14. DIAGNOSIS (ICD-10 Code/Description):		15. DATE OF SERVICE (MM/DD/YYYY) &/OR ANTICIPATED LENGTH OF CARE:	
16. CPT/HCPCS CODE &/OR DESCRIPTION OF REQUESTED SERVICES (Include units/visits, add second list page, if needed):			
17. HOW MANY VISITS HAVE OCCURRED SO FAR? (If known)		18. IS THIS A REFERRAL TO ANOTHER SPECIALTY? ☐ YES (If "YES," please fill out the Servicing Provider/Specialty information below) ☐ NO	
19. SERVICING PROVIDER'S NAME:		20. NPI #:	
21. SPECIALTY:			
22. OFFICE NAME & ADDRESS:			
23. SECURE EMAIL ADDRESS:			
24. PHONE NUMBER:		25. FAX NUMBER:	
SECTION IV: TYPE OF SERVICE REQUESTED			
26. OUTPATIENT CARE: ☐ PT ☐ OT ☐ SPEECH THERAPY		27. SURGICAL PROCEDURE: ☐ INPATIENT ☐ OUTPATIENT	
FREQUENCY & DURATION:		FACILITY NAME:	
28. ☐ IN-OFFICE PROCEDURE		29. INPATIENT CARE: ☐ LTACH ☐ ACUTE REHAB ☐ BH	
30. ☐ ADDITIONAL OFFICE VISITS (List # needed):		31. ☐ EXTENSION OF VALIDITY DATES	
32. ☐ EMERGENCY ROOM CARE		33. ☐ LABS (If done outside of office, please provide facility above)	
34. ☐ RADIOLOGY/IMAGING (If done outside of office, please provide facility above)		35. ☐ PRE-OPS LABS ☐ CHEST XRAY ☐ EKG ☐ OTHER:	
36. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).			

VETERAN'S LEGAL FULL NAME (First, MI, Last):		
SECTION V: GERIATRICS AND EXTENDED CARE SERVICES (If applicable)		
37. ☐ COMMUNITY ADULT DAY HEALTH CARE ☐ COMMUNITY NURSING HOME ☐ HOMEMAKER/HOME HEALTH AIDE ☐ HOME INFUSION ☐ HOSPICE/PALLIATIVE CARE ☐ RESPITE ☐ SKILLED HOME HEALTH CARE ☐ OTHER: _____		
FREQUENCY & DURATION: _____		
38. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		
SECTION VI: HOME OXYGEN INFORMATION (If applicable)		
39. PAO2 AT REST:	40. O2 SAT AT REST:	41. OXYGEN FLOW RATE:
42. EXTENT OF SUPPORT (Continuous, Intermittent, Specific Activity):		
43. OXYGEN EQUIPMENT (Stationary/Portable):		
44. DELIVERY SYSTEM (Cannula, Mask, Other):		
SECTION VII: DME & PROSTHETICS INFORMATION (If applicable)		
45. HCPS FOR THE ITEM(S) BEING PRESCRIBED:		
46. BRAND, MAKE, MODEL, PART NUMBERS:		
47. MEASUREMENTS:		
48. QUANTITY:	49. ICD-10:	50. PROVISIONAL DIAGNOSIS:
51. DELIVERY/PICKUP OPTIONS: ☐ DELIVER TO ORDERING PROVIDER'S ADDRESS ☐ VETERAN WILL PICKUP AT THE VA MEDICAL CENTER ☐ DELIVER TO COMMUNITY VENDOR FOR DELIVERY & SETUP FOR DME ☐ DELIVER TO VETERAN'S HOME		
SECTION VIII: DURABLE MEDICAL EQUIPMENT (DME) EDUCATION & TRAINING (If applicable)		
Please see DME Requirements/Pharmacy Requirements - Community Care (va.gov) for URGENT DME requests.		
NOTE: Failure to thoroughly complete the RFS for DME will result in delayed patient care & prevent the VA from DME fulfillment.		
52. BEFORE DME WILL BE ISSUED, EDUCATION, TRAINING, &/OR FITTING OF DME (as applicable for the specific DME being ordered) TO THE VETERAN MUST BE COMPLETE. PLEASE INDICATE WHETHER THE FOLLOWING HAS BEEN COMPLETED FOR THE VETERAN. NOTE: If not completed, DME will be mailed to requesting provider's address to coordinate an alternative time for proper instruction on DME use.		A. EDUCATION: ☐ YES ☐ NO B. TRAINING: ☐ YES ☐ NO ☐ N/A C. FITTING: ☐ YES ☐ NO ☐ N/A
53. JUSTIFICATION FOR REQUEST (To avoid delays in care, include appropriate documentation such as office notes, current treatment plans, clinical history, laboratory results, radiology results &/or medications to support the medical necessity of services requested).		

VETERAN'S LEGAL FULL NAME (First, MI, Last):	
SECTION IX: THERAPEUTIC FOOTWEAR ASSESSMENT INFORMATION (If applicable)	
54. FILL OUT THE INFORMATION BELOW (If applicable): ☐ LEFT FOOT ☐ RIGHT FOOT ☐ BILATERAL ☐ PREFABRICATED THERAPEUTIC FOOTWEAR ☐ CUSTOM THERAPEUTIC FOOTWEAR NOTE: For prescription of therapeutic footwear for severe or gross foot deformity which cannot be accommodated with conventional footwear. DESCRIBE FOOT DEFORMITY AND ADDITIONAL DETAILS:	NOTE: For prescription of therapeutic footwear due to disease pathology resulting in neuropathy or peripheral artery disease. 55. CHECK APPROPRIATE DIABETIC/AMPUTATION RISK SCORE: ☐ RISK SCORE 2: PATIENT DEMONSTRATED SENSORY LOSS (<i>inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), DIMINISHED CIRCULATION AS EVIDENCED BY ABSENT OR WEAKLY PALPABLE PULSES, FOOT DEFORMITY, OR MINOR FOOT INFECTION, & A DIAGNOSIS OF DIABETES. ☐ RISK SCORE 3: PATIENT DEMONSTRATED PERIPHERAL NEUROPATHY WITH SENSORY LOSS (<i>i.e., inability to perceive the Semmes-Weinstein 5.07 monofilament</i>), AND DIMINISHED CIRCULATION, AND FOOT DEFORMITY, OR MINOR FOOT INFECTION & A DIAGNOSIS OF DIABETES, OR ANY OF THE FOLLOWING BY ITSELF: (1) PRIOR ULCER, OSTEOMYELITIS OR HISTORY OF PRIOR AMPUTATION; (2) SEVERE PERIPHERAL VASCULAR DISEASE (PVD) (<i>intermittent claudication, dependent rubor with pallor on elevation, or critical limb ischemia manifested by rest pain, ulceration or gangrene</i>); (3) CHARCOT'S JOINT DISEASE WITH FOOT DEFORMITY; & (4) END STAGE RENAL DISEASE. NOTE: Only patients who are experiencing medical conditions noted in the risk scores can be prescribed therapeutic/diabetic footwear.
*ATTESTATION: I do hereby attest that the forgoing information is true, accurate, & complete to the best of my knowledge & I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability. I do hereby acknowledge that VA reserves the right to perform the requested service(s) if the following criteria are met: (1) The patient agrees to receive services from VA (2) Service(s) are available at VA facility & are able to be provided by the clinically indicated date (3) It is determined to be within the patients best interest. Upon completion of the requested service(s), VA will provide all resulting medical documentation to the ordering provider. If all criteria listed are not true & VA agrees the service(s) are clinically indicated, VA will provide a referral for services to be performed in the community. I do hereby attest that upon receipt of order/consult results, I will assume responsibility for reviewing said results, addressing significant findings, & providing continued care.	
56. REQUESTING PROVIDER SIGNATURE (Required):	57. TODAY'S DATE (MM/DD/YYYY):

To facilitate timely review of this request, the most recent office notes & plan of care must accompany this signed form.

For more information please visit: https://www.va.gov/COMMUNITYCARE/providers/Care_Coordination.asp.

VA Community Care Medical Policies describe standard VA health care benefit for services and procedures that community providers may recommend as necessary for a Veteran. Prior to providing care, providers should use the Community Care Medical Policies as a reference when determining if a Veteran meets VA clinical criteria. When additional services are requested, Community Care Medical Policies will be used to determine approval by a clinical reviewer. Community Care Medical Policies & supporting information can be found at: <https://www.va.gov/COMMUNITYCARE/providers/Medical-Policy.asp>